Which treatment this guide covers This guide covers National Cancer Control Programme (NCCP) 00353 osimertinib on its own: T790M-positive advanced disease, first-line activating-EGFR advanced disease, or completely resected stage IB–IIIA Ex19del/L858R adjuvant treatment. The newer licensed post-chemoradiation setting and osimertinib with platinum/pemetrexed need their own applicable plan; their study results and support medicines are not added to this monotherapy suite.

NCCP 00353 · VERSION 5

Osimertinib monotherapy

The usual starting dose is 80 mg by mouth once daily, every day without a planned rest week. Use the dose on your prescription. Advanced treatment continues while helping and tolerated; adjuvant treatment is planned for up to three years, with earlier stopping if cancer returns or side effects become unacceptable.

BASELINE → TREATMENT → FOLLOW-UP

Treatment and monitoring over time

Relative stages, not equal periods or booked appointments. Tablets are continuous; cycle 2 is a source review label. Your team confirms who arranges and reviews each test.

Check	Before the first dose	Early review / cycle 2	Ongoing daily treatment	Benefit / toxicity review	Completion or change
Treatment	Confirm adjuvant or advanced plan	Daily osimertinib; no planned rest week	Continue prescribed daily dose	Review benefit and harms	Adjuvant three-year review; advanced plan individualised
Blood tests	Blood count, liver, kidney and bone profile	Repeat at agreed reviews	Continue regular tests	Extra tests for symptoms or abnormalities	Follow unresolved changes
Heart monitoring	electrocardiogram (ECG); left ventricular ejection fraction (heart pumping measurement) (LVEF) if indicated	ECG at NCCP cycle-2 review	ECG from cycle 3 if indicated	LVEF and electrolytes as indicated	Confirm recovery before any restart
Symptoms and medicines	Review lungs, skin, bowel, eyes and interactions	Report symptoms promptly	Check adherence and tablet supply	Review toxicities and every medicine	Continue symptom advice and reproductive precautions
Cancer assessment	Confirm mutation, stage and treatment aim	Review clinical progress	Scans according to Consultant plan	Discuss response, recurrence or progression	Agree follow-up or next treatment

THE WHOLE TREATMENT JOURNEY

Osimertinib monotherapy: treatment and checks

EGFR-mutated non-small-cell lung cancer (NSCLC) · adjuvant or advanced

The usual starting dose is 80 mg by mouth once daily, every day without a planned rest week. Use the dose on your prescription. Advanced treatment continues while helping and tolerated; adjuvant treatment is planned for up to three years, with earlier stopping if cancer returns or side effects become unacceptable.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before the first dose	Early review / cycle 2	Ongoing daily treatment	Benefit / toxicity review	Completion or change
Treatment NCCP 00353 · SMPC	Confirm adjuvant or advanced plan	Daily osimertinib; no planned rest week	Continue prescribed daily dose	Review benefit and harms	Adjuvant three-year review; advanced plan individualised
 Blood tests NCCP 00353 · SMPC	Blood count, liver, kidney and bone profile	Repeat at agreed reviews	Continue regular tests	Extra tests for symptoms or abnormalities	Follow unresolved changes
Heart monitoring NCCP 00353 · SMPC	 ECG; LVEF if indicated	 ECG at NCCP cycle-2 review	 ECG from cycle 3 if indicated	LVEF and electrolytes as indicated	Confirm recovery before any restart
 Symptoms and medicines NCCP 00353 · SMPC	Review lungs, skin, bowel, eyes and interactions	Report symptoms promptly	Check adherence and tablet supply	Review toxicities and every medicine	Continue symptom advice and reproductive precautions
 Cancer assessment NCCP 00353 · SMPC	Confirm mutation, stage and treatment aim	Review clinical progress	Scans according to Consultant plan	Discuss response, recurrence or progression	Agree follow-up or next treatment

What each check is for

1 Confirm the indication

Before the first dose

Sources: NCCP 00353

2 Early heart review

Baseline and NCCP cycle 2

Sources: NCCP 00353 · SMPC

3 Possible serious toxicity

At any time

Sources: NCCP 00353 · SMPC

4 Adjuvant completion / advanced continuation

At the planned duration and benefit reviews

Sources: NCCP 00353

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.24 · Sources checked 2026-10-29

Source reconciled; independent clinical, pharmacy and patient/PPI review pending